

SUNSHINE PEDIATRIC URGENT CARE NOTE
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Date: November 05, 2024 | Department: Pediatrics

PATIENT DEMOGRAPHICS

Name: Tommy Vance
Age: 6 | Gender: Male | Weight: 22 kg | MRN: PED-44102
Parent/Guardian: Lisa Vance (Mother)

CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS

6-year-old male brought in by mother with severe right ear pain (otalgia), pulling at ear, irritability, and high fever (103.1 F) for 48 hours. Reduced oral intake.

ALLERGIES

Peanuts (Severe Anaphylaxis - carries EpiPen)
No Known Drug Allergies (NKDA)

PHYSICAL EXAMINATION

Vitals: Temp 103.1 F | HR 122 bpm | RR 26/min | SpO2 98% room air
ENT: Right tympanic membrane is bulging, erythematous, with clouding and purulent fluid behind membrane. Left TM clear.
Oropharynx: Moist mucous membranes, no exudates.

LABORATORY FINDINGS

- WBC Count: 18.5 K/uL (Reference Range: 4.5 - 13.5 K/uL for pediatrics) [HIGH]
- C-Reactive Protein (CRP): 42.0 mg/L (Reference Range: 0.0 - 5.0 mg/L) [HIGH]

ASSESSMENT / DIAGNOSIS

1. Acute Suppurative Otitis Media (Right Ear)
2. High-grade Pediatric Pyrexia

DISCHARGE MEDICATIONS & PLAN

1. Amoxicillin-Clavulanate (Augmentin) 600 mg/5 mL suspension: 9 mL PO twice daily for 10 days.
2. Children's Ibuprofen 100 mg/5 mL: 10 mL PO every 6 hours as needed for fever/pain.
3. Strict instructions: Avoid all peanut products. Follow up with Pediatrician in 5 days.